This Patient Group Direction (PGD) is intended only for registered healthcare professionals who have been named and authorised by their organisation to practice under it.

Patient Group Direction

Hepatitis A and Hepatitis B Vaccination

Havrix, Engerix B and Twinrix, for travel and lifestyle risk

Patient Group Direction, version 010, issued 30 September 2026. Supersedes Version 009, 14 September 2026.

Owner and authorising organisation: Get Real Health Limited, company number 12744898, registered office Zenvite Health Unit 23, St. Asaph Business Park, St. Asaph, Denbighshire, LL17 0LJ. This Patient Group Direction is authorised on behalf of Get Real Health Limited by its Medical Director and its Head Pharmacist, whose signatures for this version are on the authorisation page at the end of the document. It applies to the registered healthcare professionals named under it by a pharmacy or clinic that has adopted it through a Get Real Health service agreement, at the premises stated in that adoption, and to no other organisation.

What this PGD does NOT cover

These are deliberately out of scope. Each needs serology, specialist oversight or an urgent pathway that a community pharmacy cannot provide, and attempting them under this PGD would be unsafe.

- Occupational hepatitis B vaccination for healthcare workers, laboratory staff or anyone whose employment requires proof of immunity. That pathway requires pre and post vaccination serology and a structured non-responder algorithm. Refer to occupational health.
- Patients with renal failure or on dialysis, who need different products and doses (Fendrix, HBVaxPRO 40 microgram) and mandatory serology. Refer to the renal team.
- Newborns of hepatitis B positive mothers, who need a timed birth dose and often immunoglobulin. This is an antenatal pathway.
- Post-exposure prophylaxis of any kind, including needlestick injury and sexual assault. Twinrix is explicitly not recommended for post-exposure use. Refer the same day to the Health Protection Team or emergency care.
- Management of a known non-responder after a completed hepatitis B course. Refer to the GP or occupational health.
- Post-vaccination serology and its interpretation. Not available in community pharmacy and not required for the routine travel population.

Summary of Green Book chapters 17 and 18 guidance for hepatitis A and B

UKHSA, Immunisation Against Infectious Disease, chapter 17, Hepatitis A (chapter dated 12 January 2024, page updated 15 January 2024) and chapter 18, Hepatitis B (chapter dated 5 February 2026, page updated 24 February 2026). Summarised 9 September 2026.

Hepatitis A: overview

A viral infection of the liver. The disease is generally mild but severity increases with age. Asymptomatic infection is common in children; jaundice occurs in 70 to 80 per cent of those infected as adults.

There is NO chronic persistent state and chronic liver damage does not occur. Fulminant hepatitis is rare.

Spread is faecal-oral, person to person or through contaminated food or drink, including shellfish, frozen berries, dates and salad vegetables. Incubation is usually around 28 to 30 days.

Most adolescents and adults in the UK remain susceptible throughout life.

Highest risk areas for UK travellers are the Indian subcontinent, the Middle East, Africa and South East Asia, and the risk now extends to Eastern Europe.

Hepatitis A: who the chapter recommends it for

TRAVEL: recommended for those aged one year and over travelling to areas of high or medium endemicity. Give preferably at least two weeks before departure, but it can be given up to the day of departure.

Immunisation is NOT generally considered necessary for Northern or Western Europe including Spain, Portugal and Italy, nor North America, Australia or New Zealand.

People with chronic liver disease of any cause, and consider it in chronic hepatitis B or C.

People with haemophilia receiving plasma-derived clotting factors, who should be immunised SUBCUTANEOUSLY.

Gay, bisexual and other men who have sex with men, and people who inject drugs.

Occupational: laboratory workers who may be exposed, staff and residents of some large residential institutions, sewage workers at risk of repeated exposure to raw sewage, and people working with susceptible primates.

Routine immunisation is NOT indicated for most healthcare workers.

Hepatitis A: schedule

Two doses, the second 6 to 12 months after the first.

A single dose protects for up to 12 months. The second dose at 6 to 12 months gives immunity for AT LEAST 25 YEARS.

A booster 25 years after a completed course is generally not needed, except for those at ongoing risk or after exposure to a case.

A delayed second dose does not mean restarting: successful boosting occurs even when the second dose is delayed for several years.

Where rapid protection is needed in adults, a single dose of MONOVALENT vaccine is recommended. Ambirix and Havrix Junior Monodose contain more hepatitis A antigen and protect against hepatitis A more quickly than Twinrix Paediatric or Avaxim Junior.

PHENYLALANINE: Avaxim, Havrix and ViATIM contain phenylalanine in varying amounts. Advise the patient, parent or carer to account for it in meal planning on the day.

Hepatitis B: overview and who it is for

A viral infection of the liver spread by parenteral or mucosal exposure to infected blood or body fluids: sexual contact, percutaneous blood-to-blood exposure including shared injecting equipment and needlestick injury, and perinatal transmission.

Jaundice occurs in about 10 per cent of younger children and 30 to 50 per cent of adults with acute infection. Chronic infection follows in 90 per cent of those infected perinatally but 5 per cent or fewer of previously healthy adults.

TRAVEL: offer immunisation to travellers to areas of high or intermediate prevalence who will do things that place them at risk, which may include sexual activity, injecting drug use, relief healthcare work, and combat or other sports with frequent blood exposure.

Also immunise those at high risk of needing medical or dental procedures abroad where unsafe injections occur: long stays in high or intermediate prevalence areas, children and others who may need care while visiting family, people with chronic conditions who may need hospital care overseas such as haemodialysis, and those travelling for medical care.

Testing point: where testing for markers of infection is indicated, do it AT THE SAME TIME as the first dose. Do not delay vaccination waiting for results.

Hepatitis B: schedules

0, 1, 2 and 12 months, the ACCELERATED schedule, is the most commonly used and is what the chapter says should be used for most adult and childhood risk groups, because completion rates are higher.

0, 7 and 21 days plus 12 months, the SUPER-ACCELERATED or very rapid schedule, is mainly used for travel and is licensed for adults over 18 at immediate risk. A fourth dose at 12 months should be given.
0, 1 and 6 months, the STANDARD schedule, should only be used where rapid protection is not required and compliance is likely.
An interrupted primary course is RESUMED, not repeated.
Twinrix Adult at 0, 7 and 21 days gives more rapid hepatitis B protection than other schedules, but FULL hepatitis A protection comes later than with a vaccine containing a higher hepatitis A dose. A fourth dose at 12 months should be given.
Boosters: immunocompetent children and adults who completed a primary course do NOT need a reinforcing dose. Consider one for people with kidney failure, at the time of a significant exposure, and for healthcare or laboratory workers who did not respond.
Different hepatitis B monovalent and combination products are in general interchangeable.

Contraindications, pregnancy and immunosuppression

HEPATITIS A: do not give to anyone who has had a confirmed anaphylactic reaction to a previous hepatitis A-containing vaccine or to any component. Check the data sheet for excipients.
HEPATITIS B: do not give to anyone who has had a confirmed anaphylactic reaction to a previous hepatitis B-containing vaccine or to any component or manufacturing residue. The vaccine is not effective in acute hepatitis B and is not necessary where there are markers of current or past infection, but immunisation should not be delayed awaiting results.
Minor illness without fever or systemic upset is not a reason to postpone.
PREGNANCY: both may be given when clinically indicated. Hepatitis B immunisation must NOT be withheld from a pregnant woman in a high-risk category, since infection in pregnancy may cause severe maternal disease and chronic infection of the newborn.
IMMUNOSUPPRESSION: hepatitis A may be given, though seroconversion and titres may be lower and relate to CD4 count. For hepatitis B, response rates are lower depending on the degree of immunosuppression, and more doses, a higher antigen dose or a tri-antigen vaccine may improve the response.
ANTICOAGULATION: chapter 17 gives practical detail. Someone on stable anticoagulation, including warfarin where INR testing is up to date and the latest INR was below the top of range, can be vaccinated intramuscularly using a 23 gauge or finer needle, with firm pressure without rubbing for at least two minutes.

Healthcare professionals covered and training

	Healthcare professionals allowed to work under this PGD and requirements
Qualifications and Registration	• Pharmacist registered and practicing with the GPhC • Pharmacy technician registered and practicing with the GPhC
Training and assessment	Pharmacists and Pharmacy Technicians must have completed training relevant to this service to use this PGD. This training will be documented and overseen by the Get Real Health team. • All users must be familiar with the SPCs for the products they hold, and with Green Book chapters 17 and 18 • All users must be trained to the National Minimum Standards and Core Curriculum for Immunisation Training, and be competent in intramuscular vaccination • All users must be competent in the recognition and immediate management of anaphylaxis, and hold current basic life support training • All users must be able to select the correct product AND the correct strength for the age in front of them, since the paediatric formulations contain half the antigen of the adult ones • All users must be competent in travel risk assessment, including destination, duration and activity • All users must be competent in assessing consent in children and young people, including parental responsibility and Gillick competence. This PGD is available from 1 year of age, so children are in scope • All users must have completed training in working under Patient Group Directions, for example the CPPE PGD e-learning and e-assessment or an equivalent, and have been assessed as competent to work under a PGD before first working under this one; previous experience of working under a PGD is not required vaccines • All users should complete the Get Real Health training for this PGD before first working under it, or have completed equivalent training elsewhere, for example another provider's training for the same service or substantial recent experience of delivering it under a PGD, and must read this PGD in full before first working under it. The date the training was completed, and for equivalent training what it was, is to be recorded in the Get Real Health training record or in the employing organisation's own training record. The employing organisation remains responsible for being satisfied that each practitioner it authorises under this PGD is competent to do so. • Must work in compliance with the SOPs of their own employer • All users must have appropriate indemnity in place to cover this service
Further training and responsibilities	• All users must be up to date with CPD requirements and appraisal • All users must be up to date with MHRA and safety alerts with regards to medicinal products • All users must understand capacity and consent, including the Mental Capacity Act 2005, and must be able to assess consent in children and young people where this PGD covers them

Clinical condition or situation to which this PGD applies

PGD Indication	Active immunisation against hepatitis A, hepatitis B, or both, in individuals aged 1 year and over who are at increased risk through travel or lifestyle. Supply and administration under this PGD is a private service.
Inclusion criteria, hepatitis A	• Aged 1 year and over. The vaccines are not licensed below 1 year. • Travel to an area of moderate or high hepatitis A endemicity, which in practice is anywhere outside northern and western Europe, North America, Australia and New Zealand. • Or a non-travel risk factor: chronic liver disease including chronic hepatitis B or C, haemophilia or receipt of plasma derived clotting factors, people who inject drugs, gay and bisexual men and other men who have sex with men, or an occupational risk such as laboratory work or sewage work.
Inclusion criteria, hepatitis B	• Aged 1 year and over under this PGD. • Travel to an area of intermediate or high prevalence together with a risk factor, or a lifestyle risk factor regardless of travel. • Risk factors include: a longer stay or expatriate posting, likelihood of needing medical or dental care abroad, travel for medical treatment, unprotected sex with new partners, injecting drug use, tattooing, piercing or acupuncture where sterility cannot be assured, contact sports, and adopting a child from a higher prevalence country.
Inclusion criteria, both	• Valid informed consent obtained. Where the individual is under 16, from a person with parental responsibility, or from the young person where assessed as Gillick competent, with the basis of that assessment recorded. A parent accompanying a child does not automatically hold parental responsibility; ask. • The patient understands this is a private service and what it costs. • Sufficient information is available about any previous hepatitis A or B vaccination. • No exclusion criteria present.
Exclusion criteria	• Under 1 year of age. • Confirmed anaphylactic reaction to a previous dose of the same vaccine, or to any component. Havrix, Twinrix and Avaxim may contain trace neomycin, so a neomycin hypersensitivity excludes every hepatitis A containing product under this PGD. • Previous hypersensitivity reaction following a hepatitis A or hepatitis B containing vaccine. • Acute severe febrile illness. Postpone until recovered. Minor illness without fever is not a reason to defer. • Any of the situations listed under "What this PGD does NOT cover" above, in particular occupational hepatitis B vaccination, renal patients, and any post-exposure situation. • The patient requires proof of immunity, since serology is out of scope. • Under 16 and valid consent cannot be obtained from a person with parental responsibility, and the young person is not assessed as Gillick competent.
Cautions	• Pregnancy. Hepatitis A vaccine may be given where clearly indicated (Havrix preferred). Twinrix is not given in pregnancy under this PGD: where hepatitis B protection is needed in

	pregnancy, use a monovalent hepatitis B vaccine and document the risk assessment. ● Breastfeeding. Excretion in breast milk is unknown. There is no established contraindication; weigh benefit and record the decision. ● Immunosuppression, including HIV. The response may be reduced and additional doses may be needed. Where the patient needs to know whether they responded, that requires serology and is outside this PGD. Counsel and refer rather than assuming protection. ● Bleeding disorders, thrombocytopenia or anticoagulation. Intramuscular injection can usually still be given using a fine needle, 25G where possible, with firm pressure without rubbing for at least 2 minutes. Deep subcutaneous injection is the fallback, but note the Twinrix SPC warns the subcutaneous route may give a suboptimal response. ● Twinrix Paediatric and co-administration. Its SPC states that vaccines other than Cervarix should not be given at the same time. This is stricter than the general Green Book position on inactivated vaccines. Where a child needs other travel vaccines, either separate the visits or record that co-administration was an informed off-label decision. ● Latex. The presentations are described as synthetic rubber rather than natural latex, but check the current patient information leaflet for the specific product before reassuring a latex-allergic patient. ● A patient presenting too late to complete a course. See the section on late presenters below; do not simply decline.
Actions if patient is excluded or declines treatment	● Discuss the reason for exclusion and ensure the patient understands it. ● Give risk reduction advice regardless of whether vaccine is given. For hepatitis A this is food and water hygiene; for hepatitis B it is avoiding unprotected sex, unsterile tattooing, piercing and acupuncture, and not sharing needles or razors. ● Refer to the GP, occupational health, a travel clinic or the Health Protection Team as appropriate, and make the urgency clear where it is a post-exposure situation. ● Document the reason, the advice given and the decision reached.

Details of the medicines

Eight products are covered: the six in the table below, and Avaxim and Avaxim Junior, which are set out in the Avaxim section at the end of this document. Selecting the wrong strength for the age is the most likely error, because the paediatric formulations contain half the antigen of the adult ones and the packaging is similar.

Product	Age	Content	Dose	Protects against
Havrix Monodose	16 years and over	Hep A 1440 ELISA units	1.0 mL	Hepatitis A
Havrix Junior Monodose	1 to 15 years	Hep A 720 ELISA units	0.5 mL	Hepatitis A
Engerix B	16 years and over	HBsAg 20 micrograms	1.0 mL	Hepatitis B
Engerix B Paediatric	1 to 15 years	HBsAg 10 micrograms	0.5 mL	Hepatitis B
Twinrix Adult	16 years and over	Hep A 720 EU + HBsAg 20 mcg	1.0 mL	Both
Twinrix Paediatric	1 to 15 years	Hep A 360 EU +	0.5 mL	Both

		HBsAg 10 mcg		

Legal category	POM
Storage	Store at 2°C to 8°C in the original packaging, protected from light. DO NOT FREEZE. A vaccine that has been frozen, or is suspected of having been frozen, must be quarantined, labelled and not administered. Freezing separates the antigen from the aluminium adjuvant, which reduces potency and can increase reactogenicity, and it can crack the syringe. The default is to discard unless the manufacturer confirms otherwise.
Route/method of administration	• Intramuscular injection. Deltoid in older children and adults; anterolateral thigh in infants and young children. • Never into the gluteal muscle, and never intravenously or intradermally: the response is unreliable. • Where another vaccine is given at the same visit, use a separate limb where possible, or sites at least 2.5 cm apart. Record the site of each.

Schedules

Hepatitis A, monovalent (Havrix)

- A single dose gives protection from about 2 to 4 weeks, lasting at least a year.
- A booster at 6 to 12 months after the first dose gives long term protection, at least 20 to 25 years. No further boosters are recommended for immunocompetent adults.
- A late booster still works. If it is given within 5 years of the primary dose the course does not need restarting, and even beyond that a booster is generally effective rather than a reason to start again.

Hepatitis B, monovalent (Engerix B)

- Standard: 0, 1 and 6 months. All ages.
- Accelerated: 0, 1 and 2 months, with a booster at 12 months. All ages.
- Very rapid: 0, 7 and 21 days, with a booster at 12 months. Licensed for 18 years and over only. Use in a 16 or 17 year old is off-label; it may be appropriate where the alternative is no protection, but it must be an explicit decision recorded as such with the patient informed.

Combined hepatitis A and B (Twinrix)

- Twinrix Adult standard: 0, 1 and 6 months.
- Twinrix Adult rapid: 0, 7 and 21 days, with a fourth dose at 12 months. Restrict to 18 years and over under this PGD. The SPC wording and national guidance are not fully aligned on whether 16 and 17 year olds are within licence for this schedule, so the conservative position is taken; a 16 or 17 year old needing rapid protection should be treated as off-label with documented consent, or given the standard schedule.
- Twinrix Paediatric: standard 0, 1 and 6 months only. There is no accelerated or rapid schedule for the paediatric formulation. Do not improvise one.

Interrupted courses

- Resume, do not restart. A longer than recommended gap between doses does not reduce the final antibody level for either the hepatitis A or the hepatitis B component.
- Continue with the same product where possible. Monovalent hepatitis B vaccines from different manufacturers are generally interchangeable if the original is unavailable.

Late presenters, which is most of travel practice

- Hepatitis A can be given up to the day of departure. Some protection develops before antibody is detectable, so a late dose is worth giving.
- Hepatitis B needs more lead time. Even the very rapid schedule gives roughly 65% seroprotection by day 28, rising to about 76% by two months, and only reaches near-complete protection after the 12 month booster.

- For a traveller with too little time, still start the course, arrange completion on return, and be explicit that they will not be fully protected while away. Give risk reduction advice and make sure it is understood rather than just mentioned.

Anaphylaxis and observation

- Adrenaline 1:1000 injection must be immediately available whenever a vaccine is administered under this PGD, together with a written anaphylaxis protocol consistent with current Resuscitation Council UK guidance.
- All staff administering vaccines must be trained in the recognition and immediate management of anaphylaxis and hold current basic life support training.
- Observe every patient for 15 minutes after vaccination, seated, and record that the observation period was completed (see Observation after vaccination below).
- Procedures must be in place to prevent injury from a vasovagal faint.

Adverse effects, records and reporting

Adverse effects	Very common: injection site pain and redness, fatigue, headache, and in children irritability. Common: fever, malaise, injection site swelling or induration, gastrointestinal upset, drowsiness and loss of appetite. Uncommon: dizziness, myalgia, rash, influenza-like illness, vomiting. Rare or very rare: paraesthesia, arthralgia, urticaria, pruritus, lymphadenopathy, chills. Reported post-marketing: anaphylaxis, Guillain-Barre syndrome, convulsions, vasculitis, thrombocytopenia, erythema multiforme and angioedema. Consult the current SPC for the product used.
Yellow card reporting	Report suspected adverse reactions via https://yellowcard.mhra.gov.uk and inform the GP as appropriate.
Records to be kept	Record the following information: ● that valid informed consent was given, and from whom. Where the individual is under 16, the name and relationship of the person with parental responsibility, or the basis of the Gillick assessment ● name of individual, address, date of birth and GP with whom the individual is registered ● name of healthcare practitioner ● destination, departure date and the risk factors that made vaccination appropriate ● name and brand of vaccine INCLUDING the strength, batch number and expiry date ● date of administration, dose, route and anatomical site used ● which schedule was chosen, the dose number, and the date the next dose is due ● where an off-label schedule was used, that this was explained and consented to ● advice given, including risk reduction advice and advice given if excluded ● details of any adverse reaction and the action taken ● that the vaccine was administered via PGD ● that the 15 minute observation period was completed Records should be signed and dated. All records should be clear, legible and contemporaneous. The individual's GP should be informed.

Patient information

- Give the patient the manufacturer's patient information leaflet.

- Give the schedule in writing, with the date each remaining dose is due, and explain that an incomplete course gives incomplete protection.
- Be explicit about what protection they will and will not have by the time they travel, particularly for hepatitis B on a rapid schedule.
- Give risk reduction advice in every case: food and water hygiene for hepatitis A; avoiding unprotected sex, unsterile tattooing, piercing and acupuncture, and not sharing needles or razors, for hepatitis B.
- Explain that vaccination does not protect against hepatitis C, for which there is no vaccine, and that the same precautions apply.
- Counsel on the common local and systemic reactions and their self-limiting nature.
- Advise them to keep a record of what they were given, including the brand, because it determines how a course is completed elsewhere.

References

- UKHSA. Immunisation against infectious disease, the Green Book, chapter 17, Hepatitis A. Current version.
- UKHSA. Immunisation against infectious disease, the Green Book, chapter 18, Hepatitis B. Current version.
- Summary of Product Characteristics for Havrix Monodose, Havrix Junior Monodose, Engerix B, Engerix B Paediatric, Twinrix Adult and Twinrix Paediatric, current versions, electronic Medicines Compendium.
- NaTHNaC TravelHealthPro. Hepatitis A and hepatitis B factsheets and country information pages.
- National Minimum Standards and Core Curriculum for Immunisation Training.
- Resuscitation Council UK. Emergency treatment of anaphylaxis, current guidance.

Agreement to practise by the Registered Healthcare Professional

By signing this PGD you are confirming that you agree to its contents and that you will work within it. PGDs do not remove inherent professional obligations or accountability. It is the responsibility of each healthcare professional to practise only within the bounds of their own competence and professional code of conduct.

Name and address of pharmacy premises / clinic premises to which this PGD relates	

Name of healthcare professional	Job title	Registration number	Signature

Valid from date	30 September 2026

PGD adoption and authorisation by the employer/clinical lead

The employer has ensured that the person using this PGD has the knowledge and skills to safely provide the service which will encompass the administration of a vaccine.

To be completed by the adopting pharmacy: the first block is signed by that pharmacy's superintendent or clinical lead, and the second by its professional group signatory. Get Real Health signs the authorship block below, not these.

Name of superintendent / clinical lead	Job title & name of organisation	Signature	Date
Name of professional group signatory	**Job title & name of organisation**	**Signature**	**Date**

Change history to version 001

Later versions are recorded in the Version and change record at the end of this document.

Version number	Change details	Date
001	New PGD. Written because the hepatitis A and B consultation tool was live and assignable with no signed document behind it at all, the only service in the catalogue in that position, meaning every consultation was a supply with nothing authorising it. Found during the catalogue-wide audit of 21 August 2026. The tool was checked against the product SPCs first and its product and age logic is correct, so this document was written to match the tool. Scope deliberately excludes occupational hepatitis B for healthcare workers, renal and dialysis patients, newborns of hepatitis B positive mothers, all post-exposure prophylaxis, non-responder management and post-vaccination serology, because each needs serology access or specialist oversight that a community pharmacy does not have. The rapid Twinrix and Engerix B schedules are restricted to 18 and over: the SPC wording and national guidance are not fully aligned for 16 and 17 year olds, so the conservative reading is taken and off-label use requires documented consent. Twinrix Paediatric is stated as having no rapid schedule, and its SPC restriction on co-administration with vaccines other than Cervarix is flagged, because it conflicts with normal travel clinic practice.	21st August 2026

Vaccine safety requirements

The requirements on the following pages form part of this Patient Group Direction and must be met before any vaccine is administered under it.

Observation after vaccination

OBSERVE EVERY PATIENT FOR 15 MINUTES AFTER VACCINATION, SEATED.

Anxiety-related reactions including vasovagal syncope, hyperventilation and transient visual disturbance or paraesthesia can occur before or after any injection, and are commonest in adolescents. They are a response to the needle, not to the vaccine.

Have procedures in place to prevent injury from a faint.

Record that the observation period was completed.

Cold chain and excursions

Store at +2C to +8C in the original packaging to protect from light. Do not freeze, and discard any vaccine that has been frozen.

COLD CHAIN EXCURSION: any stock exposed to conditions outside +2C to +8C must be QUARANTINED and risk assessed in accordance with UKHSA Vaccine Incident Guidance before any further use.

Do not administer excursion stock until that assessment is complete and has been recorded.

Where the product SPC gives specific stability data for a temporary excursion, that data governs; otherwise quarantine and assess.

Disposal

Dispose of used syringes, needles, vials and any unused or reconstituted vaccine in a UN-approved puncture-resistant sharps container.

Follow local authority requirements and Health Technical Memorandum 07-01, Safe management of healthcare waste.

Never re-sheath a needle.

Version and change record
Version: v010
Issued: 30 September 2026
Supersedes: Version 009, 14 September 2026
Valid from: 30 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.
Changes in this version:
Training before first use, applied to every Get Real Health PGD on 30 September 2026: the staff section now states that each practitioner should complete the Get Real Health training for this PGD, or equivalent training elsewhere, before first working under it, must read the PGD in full before first use, and that the date of completion is recorded. Training is recorded; it is not a condition of access, and the employing organisation remains responsible for being satisfied of competence. No clinical content changes.

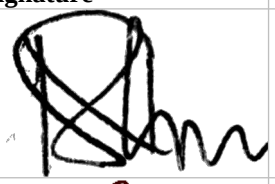
Previous version records
Version: v009
Issued: 14 September 2026
Supersedes: Version 008, 11 September 2026
Valid from: 14 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.
Changes in this version:
Identity and training, applied to every Get Real Health PGD on 14 September 2026 after two adopting pharmacies wrote: the Get Real Health logo is in the page header; the organisation that owns and authorises this direction, Get Real Health Limited, is named under the strapline with its company number and registered office; the requirement that practitioners must previously have used a PGD is replaced by a requirement for PGD training and a competence assessment, so that a newly registered pharmacist or technician who has completed the training may work under it; and the earlier change history table is headed with the versions it covers.
Previous version records
Version: v008
Issued: 11 September 2026
Supersedes: Version 007, 11 September 2026
Valid from: 11 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.
Changes in this version:
Decision 6: the estate's standard consent in children and young people block was added, in the wording used by the rabies PGD: a training requirement to assess consent in children including parental responsibility and Gillick competence, an inclusion criterion that consent for a patient under 16 comes from a person with parental responsibility or from the young person where assessed as Gillick competent with the basis recorded, a matching exclusion where neither is available, and a records requirement for the name and relationship of the consenting adult or the basis of the Gillick assessment; the consultation tool offers Gillick competence from 12 to 15 years.
Decisions of the authorising signatories, 11 September 2026: the questions raised by the reviews of 10 and 11 September were put to the Medical Director and answered; each answer is written into this document above and, where the answer set a rule, into the consultation tool. Text drafted from a source for the Head Pharmacist to confirm is marked as such in the change lines.
Previous version records
Version: v007
Issued: 11 September 2026
Supersedes: Version 006, 11 September 2026
Valid from: 11 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.
Changes in this version:
The consent criterion and the records row now define the child: a patient aged 16 or over consents for themselves; under 16, consent comes from a person with parental responsibility.
The neomycin sentence now covers Avaxim as well as Havrix and Twinrix, so a neomycin hypersensitivity excludes every hepatitis A containing product under this PGD.
The pregnancy caution no longer leaves a case for Twinrix in pregnancy: where hepatitis B protection is needed, a monovalent hepatitis B vaccine is used.
The products sentence now counts eight products and points to the Avaxim section added at version 003.

The anaphylaxis and observation row now states the 15 minute seated observation that the safety block requires, and the records row asks for it to be recorded.
Wording corrections from the tool alignment and adversarial review of the consultation tools, 11 September 2026. Each correction resolves a contradiction inside the document or a plain error, taking the safer reading; no indication is widened, no dose raised and no exclusion removed. Questions that need a clinical decision are recorded separately and are not changed here.
Previous version records
Version: v006
Issued: 11 September 2026
Supersedes: Version 005, 10 September 2026
Valid from: 11 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.
Changes in this version:
The 'Why this document exists' narrative is removed from the cover. The incident it described is recorded in Get Real Health's governance log, not in the PGD.
Structural clean-up applied to every live Get Real Health PGD on 11 September 2026, following the adversarial review of 10 September: the adopting pharmacy's authorisation block is blank, for the pharmacy's own signatories; one signed authorisation page for this version replaces the earlier signature blocks; every validity cell states this version's dates (valid from 11 September 2026, expiry 31 July 2027); narration about earlier versions, the consultation tool and individual customers is removed from the body of the document; the change records of earlier reissues are carried below as previous version records. No clinical criterion changes unless listed.
Previous version records
Version: v002
Issued: 9 September 2026
Supersedes: the previous signed version of this document.
Change: the vaccine safety requirements above were added following an estate-wide sweep of every vaccination PGD, which found that the signed documents were missing, between them, any requirement for adrenaline to be available, any anaphylaxis provision, a stated observation period, a cold chain excursion procedure, a sharps disposal provision, a batch number requirement, and any wording on consent in children and young people. The specific items added to THIS document are: Observation after vaccination, Cold chain and excursions, Disposal.
Authorised by Nitin Shori, Medical Director (GMC 6047293) and Chris Pilkington, Head Pharmacist (GPhC 2046322), on 9 September 2026. Signatures applied digitally on their joint instruction, which is the established process for Get Real Health PGDs.
AVAXIM: HEPATITIS A VACCINE ADDED AT VERSION 003, 9 SEPTEMBER 2026
Avaxim and Avaxim Junior (Sanofi) are now AUTHORISED under this PGD as monovalent hepatitis A vaccines, alongside Havrix Monodose and Havrix Junior Monodose. Version 002 authorised Havrix only, so a pharmacy holding Avaxim, which is a widely stocked UK-licensed hepatitis A vaccine, could not use it. Raised twice by an adopting pharmacy.
AVAXIM, 160 EU per 0.5 mL dose, PL 23228/0006. Licensed for adults and adolescents 16 YEARS AND OVER. Not recommended at 15 years or under: use Avaxim Junior.
AVAXIM JUNIOR, 80 EU per 0.5 mL dose, PL 23228/0007. Licensed from 1 YEAR UP TO AND INCLUDING 15 YEARS.
DOSE AND ROUTE, both products: 0.5 mL by intramuscular injection. Deltoid in adults, older children and adolescents; anterolateral thigh in young children. Never intradermally, never intravascularly, and not into the buttock. In thrombocytopenia or where there is a bleeding risk the subcutaneous route may be used.
SCHEDULE, AVAXIM adult: a single dose, with protective antibody not reached until about 14 days after it. A booster dose for long-term protection is preferably given between 6 and 12 months after the first, and may be given up to 36 months after it.
SCHEDULE, AVAXIM JUNIOR: a single dose, with initial protection from about 2 weeks. The booster is given between 6 months and 15 years after the first dose. Note that this window is much wider than the adult one; do not carry the adult 36 month limit across to a child.
INTERCHANGEABILITY: Avaxim may be used as the booster in a person aged 16 or over who had a different inactivated hepatitis A vaccine, monovalent or combined with Vi polysaccharide typhoid, 6 to 36 months previously. Avaxim Junior may likewise be used as a booster after another inactivated hepatitis A vaccine.
CONTRAINDICATIONS, both products: hypersensitivity to any active substance or excipient, or to NEOMYCIN, which may be present in trace amounts; hypersensitivity following a previous injection of this vaccine; and acute severe febrile illness, in which case delay.

PHENYLALANINE: each 0.5 mL dose of either product contains 10 micrograms of phenylalanine. Relevant only in phenylketonuria, and at that quantity almost certainly immaterial, but ask if the patient raises it.
LATEX: the needle shield of the attached-needle presentation of adult AVAXIM may be natural rubber. Check the presentation in hand before use in a latex-sensitive patient. The Avaxim Junior needle shield is polyisoprene.
PREGNANCY: AVAXIM should not be used in pregnancy unless clearly necessary, following an assessment of risks and benefits, and that assessment must be recorded. This is a more cautious position than Havrix; where hepatitis A vaccine is clearly indicated in pregnancy, Havrix remains the preferred choice under this PGD.
BREASTFEEDING: both Avaxim products may be used during breastfeeding.
IMMUNOSUPPRESSION: the response may be impaired. Vaccination of a person with chronic immunodeficiency including HIV is still recommended, but the antibody response may be limited and post-course serology should be considered. Serology is out of scope of this PGD; refer for it.
CO-ADMINISTRATION: do not mix Avaxim with any other vaccine in the same syringe. Give other vaccines at a different site with a different syringe and needle. Seroconversion is not affected by concurrent Vi polysaccharide typhoid or yellow fever vaccine given at a separate site.
STORAGE: 2C to 8C, do not freeze, discard if frozen, keep in the original package to protect from light. NOTE: neither Avaxim SPC contains any temperature excursion stability data, so there is no permitted out-of-fridge period for these products. Any excursion means quarantine and risk assessment under UKHSA Vaccine Incident Guidance before further use.
CHANGE RECORD, version 003, 9 September 2026: Avaxim and Avaxim Junior added as authorised hepatitis A vaccines. Havrix, Engerix B and Twinrix are unchanged and remain authorised. Nothing else in this document is altered. This reissue is not a clinical review of the remainder of the PGD. Authorised by Nitin Shori (GMC 6047293) and Chris Pilkington (GPhC 2046322), signatures applied digitally on their joint instruction.
Version: v004
Issued: 10 September 2026
Supersedes: Version 003, 9 September 2026
Change: the summary of the governing guidance is added as part 2 of the document, between the cover and the PGD, which is the house format for every Get Real Health PGD. It is summarised from the source it names and adds nothing to it. Nothing else in this document is altered, and this is not a clinical review of the remainder.
Version: v005
Issued: 10 September 2026
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Valid from: 10 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.
Changes in this version:
Validity: this version is valid from 10 September 2026 and expires on 31 July 2027. The signed master previously stated no period of validity, or an expiry of 31 October 2026; those are superseded. No expiry date was stated in any version.

Signed on behalf of Get Real Health

Version v010, 30 September 2026.

Name	Qualification	Signature	Date
Nitin Shori	Medical Director GMC: 6047293		30 September 2026
Chris Pilkington	Head Pharmacist GPhC: 2046322		30 September 2026

Both authorising signatories reviewed this version together on 30 September 2026 and gave their authorisation for it to be issued. Signatures were applied digitally on their joint instruction, which is the established process for Get Real Health PGDs. This version is not valid without both signatures.